



HYPERSOR TABLET 100mg

Description: White, round and top-scored tablet. Diameter = 10.00mm.

Content

Metoprolol Tartrate U.S.P. 100mg/tablet

Pharmacology

Metoprolol is a cardioselective beta-1-adrenergic blocker lacking intrinsic sympathomimetic activity. It has both antianginal and antihypertensive properties. Its antianginal effect is probably due to reduced myocardial oxygen requirements and antihypertensive effect may include reduced cardiac output, decreased sympathetic outflow to peripheral vasculature, and inhibition of renin release by the kidney. It is completely absorbed orally, subject to considerable first-pass metabolism, and excreted in the urine.

Indications

For treatment of angina pectoris and hypertension, and prophylactic treatment of myocardial reinfarction.

Adverse Effects

The most common side effects appear to be lassitude, GI disturbances (nausea, vomiting or abdominal pain) and disturbances of sleep pattern. Cardiovascular effects which have been reported occasionally are bradycardia, postural hypotension and rarely, heart failure, palpitations, cardiac arrhythmias, Raynauds phenomenon, peripheral oedema and precordial pain. Skin rashes (urticaria, psoriasiform, dystrophic skin lesions) and positive anti nuclear antibodies (not associated with SLE) occur rarely. Isolated cases of photosensitivity, increased sweating and alopecia have been reported. Respiratory effects include occasional reports of dyspnoea on exertion and rare reports of bronchospasm and isolated cases of rhinitis. Isolated cases of weight gain, thrombocytopenia, disturbances of vision, conjunctivitis, tinnitus and dry or irritated eyes have also been reported. Mental depression, decreased sexual ability, dizziness, weakness, headache, fast or irregular heartbeat, sudden shortness of breath, sweating and trembling may occur occasionally.

Precautions

- Metoprolol may aggravate bradycardia, symptoms of peripheral arterial circulatory disorders and anaphylactic shock.
- Caution in patients with history of allergy or mental depression, bronchial asthma, congestive heart failure, hypoglycaemia, severe hepatic or renal function impairment.
- Check with physician before discontinuing medication; gradual dosage reduction may be necessary.
- Caution when driving or doing jobs requiring alertness because of possible drowsiness or dizziness.
- Patients receiving beta-blockers who display symptoms of cardiac failure should be digitalized and/or receive a diuretic.
- Safety for use in pregnancy and lactation has not been established.

Contraindications

Avoid in patients with overt cardiac failure; cardiogenic shock, heart block, 2nd or 3rd degree atrioventricular (AV) block; or sinus bradycardia (heart rates less than 45 beats per minute).

Severe peripheral arterial disease.

Known hypersensitivity to metoprolol or other β -blockers.

Metoprolol is also contraindicated when myocardial infarction is complicated by significant bradycardia, first degree heart block, systolic hypotension (<100mmHg) and/or severe heart failure.

Drug Interactions

- Concurrent use with general anaesthetics may increase the risk of myocardial depression and hypotension.
- Co-administration of calcium channel blocking agents or clonidine may potentiate antihypertensive effects.
- Concomitant therapy with sympathomimetic amines having beta-adrenergic stimulant activity may result in mutual inhibition of therapeutic effects.
- Cimetidine may increase metoprolol effect because of increased blood concentration resulting from inhibition of hepatic enzymes and reduced hepatic blood flow.

Overdoseage

Signs of overdose may include sudden bradycardia, hypotension, bronchospasm, cardiac failure, peripheral cyanosis, and seizures, for treatment of overdose, in addition to gastric lavage, supportive and symptomatic therapy are necessary.

Dosage and Administration

- Adults - Antianginal : Oral, usually metoprolol 50 - 100mg twice or three times daily.
Antihypertensive : Oral, initially 100mg a day in single or divided doses, the dosage being increased at one week intervals as needed and tolerated up to a total of 450mg a day if necessary.

Note: In order to maintain satisfactory blood pressure control, some patients may require the total daily dose to be divided into three separate doses.

- Myocardial reinfarction prophylactic - Early treatment: Oral, 50mg every 6 hours starting 15 minutes after the last intravenous dose as soon as clinical condition allows, for patients who tolerate the full intravenous dose (a dose of 25 to 50mg may be used for patients who do not). This dosage is continued for 48 hours, followed by:

- Late treatment: Oral, 100mg two times a day for at least 3 months and possibly for as long as 1 to 3 years.

Children - Dosage has not been established.

Storage : Keep container tightly closed. Store in a cool, dry place away from light, below 30°C.

Presentation/Packing : Tablet 100mg x 1000's (blister).

Shelflife : 3 years

Manufactured by : **PRIME PHARMACEUTICAL SDN. BHD.**

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